

D343

Foundations of Advanced Psychiatric Mental Health Practice

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

Method © 2026 Erik Daniel Lindquist — Course Mastery Coach, armored drill. Items drafted with AI assistance under his direction.

How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A 34-year-old teacher who moved to a new school district in August reports three weeks of depressed mood nearly every day, poor sleep, low energy, trouble concentrating, and feelings of worthlessness. She has missed several workdays. Labs, including thyroid, are normal and she uses no substances. She has never had a manic or hypomanic episode. Which conclusion fits DSM-5-TR criteria?

- A. It is an adjustment disorder because she moved in August.
- B. She needs all nine symptoms to meet criteria.
- C. She does not meet criteria because symptoms must last at least 6 months.
- D. She meets criteria for a major depressive episode.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A patient has had depressed mood, insomnia, and poor appetite nearly every day for four weeks with clear impairment at work. No other symptoms are present. What is missing for a major depressive episode?

- A. At least two more of the nine symptoms, for a total of five.
- B. A longer duration of at least 6 months.
- C. A history of at least one manic episode.
- D. Nothing; depressed mood and impairment are enough.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A 22-year-old college senior whose mother has bipolar I disorder develops his first manic episode during final exams while sleeping four hours a night. He is majoring in engineering and lives off campus. Which model best explains the timing of onset?

- A. Purely genetic: stress plays no role.
- B. Caused by the demands of an engineering major.
- C. Purely environmental: anyone would become manic under exam stress.
- D. Stress-diathesis: a genetic vulnerability triggered by stress and sleep loss.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

During a session, a patient who works night shifts at a warehouse says he has bought a gun and plans to shoot his former supervisor, whom he names, this weekend. He has been in therapy for eight months. What is the PMHNP's obligation?

- A. Document the threat and review it at the next session.
- B. No duty exists because the relationship is long-standing.
- C. Take protective action required by state law, such as warning the supervisor, notifying police, or seeking hospitalization.
- D. Keep the statement confidential to protect the therapeutic alliance.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

A patient angrily says, "People like my old boss make me sick," with no plan, weapon, or intent to harm anyone. What is the most appropriate response?

- A. End treatment because of hostility.
- B. Explore the anger therapeutically and assess risk; no duty-to-warn action is triggered.
- C. Immediately notify police and the former boss.
- D. Begin involuntary commitment.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A voluntarily admitted patient with schizophrenia, who enjoys painting in the day room, declines a proposed knee surgery. She explains the risks and benefits back accurately and says she would rather use physical therapy. Which statement is correct?

- A. Her hobbies show she is too distracted to decide.
- B. A schizophrenia diagnosis means she lacks capacity.
- C. She has capacity for this decision, and her refusal should be respected.
- D. The PMHNP must first declare her legally incompetent.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

An adult with bipolar disorder, who lives with his sister and works from home, stops his mood stabilizer. He is sleeping normally, eating, paying bills, and denies thoughts of harming himself or others. His sister asks for involuntary hospitalization. What is the appropriate position?

- A. He does not meet criteria; offer outpatient follow-up and education in the least restrictive setting.
- B. Hospitalize anyone with bipolar disorder who stops treatment.
- C. Commit him because a family member requested it.
- D. Commit him because he stopped his medication.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A recently widowed woman who immigrated from the Caribbean 20 years ago says she hears her late husband's voice comforting her at night, which her church community sees as normal. She works as a bank teller and functions well. What is the best interpretation?

- A. Brief psychotic disorder because she hears a voice.
- B. A culturally and religiously normative grief experience; explore its meaning with a cultural formulation rather than diagnose psychosis.
- C. Stress from bank work causing hallucinations.
- D. Schizophrenia, because any voice-hearing is psychotic.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A caregiver has managed her husband's dementia for two years. She has adjusted her routine and copes day to day, but her cortisol levels remain elevated. She recently started a book club. Which stage of the general adaptation syndrome is she in?

- A. Exhaustion
- B. Recovery, because she joined a club
- C. Alarm
- D. Resistance

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10. [TRANSFER — reversed stem / different variable]

After years of chronic stress, a man develops frequent infections, fatigue, and inability to cope. Which stage of the general adaptation syndrome does this reflect?

- A.** Exhaustion
- B.** Homeostasis
- C.** Resistance
- D.** Alarm

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

After being criticized by his manager in a meeting, a man drives home, then yells at his teenage son for leaving shoes by the door. He drives a pickup truck and coaches youth soccer. Which defense mechanism is he using?

- A.** Sublimation, through coaching soccer
- B.** Projection
- C.** Displacement
- D.** Denial

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

A man who is attracted to a coworker accuses his wife of wanting to cheat on him. Which defense mechanism is this?

- A.** Displacement
- B.** Projection
- C.** Reaction formation
- D.** Rationalization

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer D Concept 1

A 34-year-old teacher ~~who moved to a new school district in August~~ reports three weeks of depressed mood nearly every day, poor sleep, low energy, trouble concentrating, and feelings of worthlessness. She has missed several workdays. Labs, including thyroid, are normal and she uses no substances. She has never had a manic or hypomanic episode. Which conclusion fits DSM-5-TR criteria?

Target word: does she meet MDE criteria **Decides:** Five symptoms including depressed mood, over 2+ weeks, with impairment and medical causes excluded.

A	Camouflage	When full MDE criteria are met, MDE takes precedence over adjustment disorder.
B	Absolute language	Five of nine is required.
C	Confusion set	Six months is the GAD duration; MDE needs 2 weeks.
D	Correct	Five symptoms incl. depressed mood for three weeks, with impairment, not explained by other causes.

2. Answer A Concept 1 • transfer

A patient has had depressed mood, insomnia, and poor appetite nearly every day for four weeks with clear impairment at work. No other symptoms are present. What is missing for a major depressive episode?

Target word: reverse: what fails **Decides:** Only three symptoms are present; five are required.

A	Correct	Duration and impairment are met; symptom count is not.
B	Confusion set	Two weeks is enough for MDE.
C	Inverted operation	A manic history would rule out unipolar MDD, not complete it.
D	Dropped element	Ignores the five-symptom minimum.

3. Answer D Concept 2

A 22-year-old college senior whose mother has bipolar I disorder develops his first manic episode during final exams while sleeping four hours a night. ~~He is majoring in engineering and lives off campus.~~ Which model best explains the timing of onset?

Target word: model of onset **Decides:** Inherited vulnerability plus acute stress.

A	Dropped element	Ignores the clear stressor.
B	Camouflage	The major is incidental detail, not a mechanism.
C	Absolute language	Most students under stress do not develop mania.
D	Correct	Both a predisposition and a stressor are present.

4. Answer C Concept 3

During a session, a patient ~~who works night shifts at a warehouse~~ says he has bought a gun and plans to shoot his former supervisor, whom he names, this weekend. ~~He has been in therapy for eight months.~~ What is the PMHNP's obligation?

Target word: duty to protect **Decides:** A credible threat against an identifiable person.

A	Dropped element	Leaves out timely protective action.
B	Camouflage	Length of treatment does not change the duty.
C	Correct	Tarasoff-type duties apply to serious threats against identifiable victims.
D	Inverted operation	The duty to protect overrides confidentiality here.

5. Answer B Concept 3 • transfer

A patient angrily says, “People like my old boss make me sick,” with no plan, weapon, or intent to harm anyone. What is the most appropriate response?

Target word: reverse: no duty triggered **Decides:** No serious threat or specific intent.

A	Inverted operation	Abandons care instead of addressing anger.
B	Correct	Vague anger without intent or plan does not meet the threshold.
C	Absolute language	Over-applies the duty and breaches confidentiality.
D	Adjacent category	No danger criteria are met.

6. Answer C Concept 4

A voluntarily admitted patient with schizophrenia, ~~who enjoys painting in the day room,~~ declines a proposed knee surgery. She explains the risks and benefits back accurately and says she would rather use physical therapy. Which statement is correct?

Target word: capacity for this decision **Decides:** She understands, appreciates, reasons, and expresses a choice.

A	Camouflage	Painting has no bearing on capacity.
B	Absolute language	Diagnosis alone does not remove capacity.
C	Correct	Capacity is decision-specific and she meets the elements.
D	Confusion set	Competence is a court determination; capacity is clinical.

7. Answer A Concept 5

An adult with bipolar disorder, ~~who lives with his sister and works from home,~~ stops his mood stabilizer. He is sleeping normally, eating, paying bills, and denies thoughts of harming himself or others. His sister asks for involuntary hospitalization. What is the appropriate position?

Target word: commitment criteria **Decides:** No danger or grave disability.

A	Correct	Refusing medication alone does not justify commitment.
B	Absolute language	Ignores individual assessment.
C	Adjacent category	Family may petition, but criteria still must be met.
D	Inverted operation	Non-adherence without danger is not a legal basis.

8. Answer B Concept 6

A recently widowed woman who immigrated from the Caribbean 20 years ago says she hears her late husband’s voice comforting her at night, which her church community sees as normal. ~~She works as a bank teller~~ and functions well. What is the best interpretation?

Target word: pathology or cultural norm **Decides:** The experience is culturally normative and non-impairing.

A	Dropped element	Ignores cultural context and intact function.
B	Correct	Shared beliefs in context are not pathology.
C	Camouflage	Her job is irrelevant.
D	Absolute language	Not all voice-hearing is pathological.

9. Answer D Concept 7

A caregiver has managed her husband's dementia for two years. She has adjusted her routine and copes day to day, but her cortisol levels remain elevated. ~~She recently started a book club.~~ Which stage of the general adaptation syndrome is she in?

Target word: GAS stage **Decides:** Long-term coping with sustained arousal.

A	Adjacent category	Exhaustion means coping resources are depleted; she is still coping.
B	Camouflage	A new hobby does not end the stress response.
C	Inverted operation	Alarm is the initial fight-or-flight reaction.
D	Correct	Ongoing adaptation with sustained stress hormones.

10. Answer A Concept 7 · transfer

After years of chronic stress, a man develops frequent infections, fatigue, and inability to cope. Which stage of the general adaptation syndrome does this reflect?

Target word: reverse: from signs to stage **Decides:** Depleted resources and illness.

A	Correct	Resources are depleted and illness risk rises.
B	Confusion set	Not a GAS stage, and function is not restored.
C	Adjacent category	He is no longer adapting.
D	Inverted operation	Alarm is the brief first reaction.

11. Answer C Concept 8

After being criticized by his manager in a meeting, a man drives home, then yells at his teenage son for leaving shoes by the door. ~~He drives a pickup truck and coaches youth soccer.~~ Which defense mechanism is he using?

Target word: which defense **Decides:** His own anger redirected to a safer target.

A	Camouflage	Coaching is unrelated to this episode.
B	Confusion set	Projection assigns one's own feelings to someone else.
C	Correct	Anger at the manager is redirected to the son.
D	Adjacent category	He is not refusing to accept reality.

12. Answer B Concept 8 · transfer

A man who is attracted to a coworker accuses his wife of wanting to cheat on him. Which defense mechanism is this?

Target word: reverse: projection vs. displacement **Decides:** His own feeling is attributed to someone else.

A	Confusion set	Displacement redirects a feeling to a safer target; it does not reassign ownership.
B	Correct	He assigns his own unacceptable wish to his wife.
C	Adjacent category	Reaction formation shows the opposite feeling.
D	Inverted operation	Rationalization justifies one's own behavior rather than blaming another's motives.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	6	
Confusion set	Every option is real; the test is exact role-matching.	6	
Camouflage	Answers a stem fact that was never the question.	7	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	DSM-5-TR major depressive episode criteria	Five or more of nine symptoms over the same 2-week period, including depressed mood or loss of interest, causing significant distress or impairment and not explained by a substance or medical condition.	Diagnosing on symptom count alone, or with too few symptoms or too short a duration.	A symptom list with a duration.
2	Stress-diathesis (vulnerability-stress) model	Disorders emerge when an underlying vulnerability (genetic, biological, psychological) interacts with enough environmental stress.	Attributing illness to a single cause, either genes or stress alone.	A family history plus a recent stressor.
3	Duty to warn / protect	When a patient communicates a serious threat of violence against a reasonably identifiable person, the clinician must take protective action per state law (e.g., warn the victim, notify police, hospitalize).	Believing confidentiality always outweighs a specific threat, or that vague anger triggers the duty.	A named or identifiable target and a credible threat.
4	Decision-making capacity	Capacity is a clinical, decision-specific judgment (understand, appreciate, reason, express a choice); competence is a legal determination by a court.	Assuming a psychiatric diagnosis or involuntary status means no capacity for any decision.	A patient with mental illness refusing or accepting a specific treatment.
5	Least restrictive alternative and involuntary commitment	Involuntary hospitalization requires mental illness plus danger to self or others or grave disability, and care should use the least restrictive setting that is safe.	Committing someone because they refuse medication or have an unusual lifestyle.	A refusal of care without imminent danger.
6	Culture and diagnosis	Experiences that are normative in a person's cultural or religious context are not pathology; the DSM-5-TR Cultural Formulation Interview helps clarify meaning.	Labeling culturally expected beliefs or grief practices as psychosis.	A belief or behavior shared by the patient's community.
7	General adaptation syndrome	Stress response moves through alarm (fight or flight), resistance (adaptation with sustained cortisol), and exhaustion (resources depleted, illness risk rises).	Placing chronic, long-term coping in the alarm stage.	The time course of a stress response.

#	Concept	Rule	Trap	Tell
8	Defense mechanisms	Displacement redirects feelings to a safer target; projection attributes one's own unacceptable feelings to others; denial refuses to accept reality; rationalization supplies acceptable reasons.	Confusing displacement with projection.	Who the feeling belongs to and where it is aimed.

Course scope (public description): Foundations of Advanced Psychiatric Mental Health Practice guides students through the process of learning about mental health and mental illness. This course presents the history of psychiatric care, along with cultural components that influence individual attitudes and behaviors. This course introduces conceptual models and theories related to practice that provide the basis for understanding the development of psychopathology to apply appropriate therapeutic strategies. This course includes clinical practice guidelines using the DSM-5-TR (Diagnostic Statistical Manual of Mental Disorders) as a basis for diagnostic consistency across the lifespan. This course also includes relevant advanced practice issues, legal and ethical components, and barriers to practice that a mental health psychiatric nurse practitioner may encounter. Various psychological responses to stress are also discussed. The following courses are prerequisites: All MSN Core courses, Advanced Pathophysiology for the Advanced Practice Nurse, Advanced Pharmacology for the Advanced Practice Nurse, and Advanced Health Assessment for the Advanced Practice Nurse.